

City Hospital Diabetes Centre

Date: 09 Feb 2024

CLINIC LETTER

Patient: Robert Patel

DOB: 1964-11-03 (age 59)

NHS Number: 999 600 0006

Dear Dr Mehta,

Thank you for referring Mr Patel for treatment escalation. I reviewed him today in the diabetes clinic.

History: Type 2 diabetes diagnosed 2018, established on metformin 1 g BD since 2018. HbA1c trajectory: 7.4% (Nov 2021), 8.1% (Nov 2022), 8.6% (Nov 2023). Patient reports good adherence; no hypoglycaemic episodes; no GI side effects from metformin.

Cardiovascular and renal status: Hypertensive on ramipril, target now achieved per GP increase to 10 mg OD. eGFR 68 (stable). No proteinuria. No known cardiovascular disease.

Examination: BP 132/78, HR 76. Weight 95 kg (down 1 kg from GP review). Foot examination: pulses intact, monofilament intact, no ulceration. BMI 32.0.

Investigations (today): HbA1c 8.7% (worsening), eGFR 65, ACR 2.4 mg/mmol.

Allergies: NKDA.

Assessment: Persistently suboptimal glycaemic control on metformin monotherapy. BMI in obese range. Cardiovascular risk profile and renal function support SGLT2 inhibitor as second-line agent per NICE NG28 update 2022. Will initiate dapagliflozin.

Plan:

- **Start dapagliflozin 10 mg OD from 12 Feb 2024** — counselled re: DKA risk, sick-day rules, urogenital infection awareness, importance of hydration
- Continue metformin 1 g BD
- Continue ramipril 10 mg OD
- Repeat HbA1c, U&E, urinalysis in 3 months
- Diabetes clinic follow-up at 6 months
- Patient leaflet on SGLT2 inhibitor side-effects provided

Yours sincerely,

Dr K. Adeyemi, Consultant Diabetologist